

BACKGROUND

Working as an emergency nurse, the resuscitation area is fast-paced, intense and demands constant vigilance. The role requires advanced clinical skills, excellent communication and the ability to juggle several responsibilities at once. Within this role, is the completion and documentation of observations – as highlighted by the RCEM guidelines, “initial observations (should be completed) within the first 15 minutes of a patient’s arrival” (RCEM Emergency Department Standards: Quality in Emergency care, 2022). This time frame is aimed across our care in the emergency department but is crucial for resus patients to ensure appropriate triage and clinical decision-making.

AIM

The aim of this project is to evaluate whether initial observations and their documentation are being completed within the first 15 minutes of arrival to the resuscitation area. Both qualitative and quantitative data have been collected and analyzed to assess compliance and identify areas for improvement.

METHODOLOGY

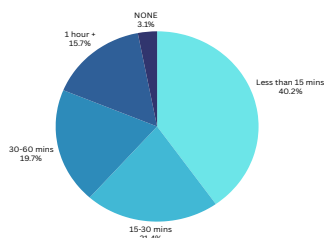
This project used both quantitative and qualitative methods.

The **quantitative data** was gathered through an audit of 223 resus patient records from 1 January 2025 to 16 January 2025. **Qualitative data** was collected through surveys of 86 band 5 nurses (47.6% response rate) and 54 healthcare assistants (55.5% response rate) to explore barriers to timely documentation and support needs for resus.

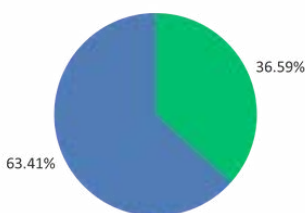
RESULTS

QUANTITATIVE RESULTS

Across the 2-week data investigated, less than half (40.2%) of all resus patients received observations within 15 minutes of arrival, next majority was 15-30mins at 21.4%. Over an hour waiting for observations, 16% and 7 patients did not receive any observations whilst in the resus area.



The data reveals that 63% of staff nurses do not believe the 15-minute target is being met.

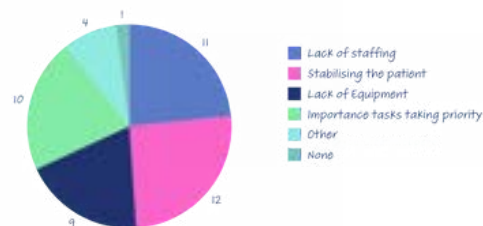


Nursing staff suggested two key solutions: adding more staff to resus to improve observation targets and efficiency, and implementing synchronized vital monitoring.

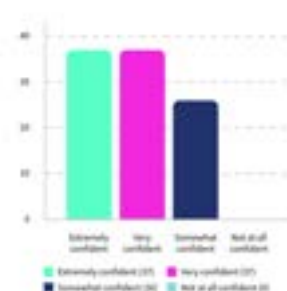


QUALITATIVE RESULTS

It identified four main factors preventing staff from documenting initial observations: insufficient staffing, the need to stabilize the patient, lack of equipment, and the prioritization of other tasks.



HCA responses indicated a strong willingness to support the resus team, provided that a competency pack is introduced before allocation.



CONCLUSION

Nurses in the resuscitation area expressed concerns that initial observations were not completed within the national 15-minute target. Data collected from January 1 to 16, 2025, confirmed that less than 50% of patients had documented observations within this timeframe. This project highlights a known issue, but for the first time, it uses real data from our own practice and includes direct input from staff. The results have been presented to the matrons to raise awareness of the problem and provide a foundation for future solutions.

REFERENCES

RCEM Emergency Department Standards: Quality in Emergency care, 2022
[Emergency Department Standards 2020.pdf](#) (Accessed 28/2/25)

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